



UNIVERSITY OF EASTERN AFRICA BARATON

SCHOOL OF NURSING AND HEALTH SCIENCES

DEPARTMENT OF NURSING

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FOUNDATION

COURSE CODE: NRSG215

TOPIC: NURSING THEORIES

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1. Imogene King's Theory of Goal Attainment

Background:

Imogene King from 1923-2007 was an American nurse theorist who developed the Theory of Goal Attainment in the 1960s. Her work was influenced by general systems theory and focused on nurse-patient interactions to achieve health goals.

Major Concepts:

1. Personal System that is individual perceptions
2. Interpersonal System that is nurse-patient interactions
3. Social System that is broader societal influences

Assumptions:

1. Nurse-patient communication is essential
2. Patients are active participants in care
3. Health is a dynamic state influenced by interactions

Application to Nursing Process:

1. **Assessment:** Nurse evaluates patient's perception of health
2. **Planning:** Mutual goal-setting e.g., pain management plan
3. **Implementation:** Collaborative actions to achieve goals
4. **Evaluation:** Determine if goals were met

Example in Practice:

For a diabetic patient, the nurse would:

1. Assess understanding of diabetes management
2. Set mutual goals for blood sugar control
3. Teach meal planning and medication adherence
4. Evaluate progress at follow-up visits

2. Jean Watson's Theory of Human Caring

Background:

Jean Watson developed the Theory of Human Caring in 1979. Her work integrates humanistic, spiritual and scientific aspects of nursing.

Major Concepts:

1. Caritas Processes that is love/kindness in care
2. Transpersonal Caring Relationships
3. Caring Moments

Assumptions:

1. Caring is the essence of nursing

2. Health involves mind-body-spirit harmony
3. Caring promotes healing

Application to Nursing Process:

1. **Assessment:** Identify emotional/spiritual needs
2. **Planning:** Incorporate compassionate interventions
3. **Implementation:** Use therapeutic presence
4. **Evaluation:** Assess emotional/spiritual well-being

Example in Practice:

For a terminal cancer patient:

1. Assess fears about dying
2. Plan spiritual support interventions
3. Provide active listening and arrange chaplain visit
4. Evaluate peace of mind

Comparison of Imogene King's Theory of Goal Attainment and the Adventist Whole Person Care Nursing Model

1. Philosophical Foundations

Aspect	King's Theory of Goal Attainment	Adventist Whole Person Care Model
Theoretical Basis Systems	Systems theory and interpersonal relationships	Biblical principles, holistic health
Primary Focus	Nurse-patient interactions for goal achievement	Spiritual, physical, and mental well-being
View of Health	Dynamic state influenced by interactions	Harmony of body, mind, and spirit

2. Core Concepts

Concept	King's Theory	Adventist Model
Patient Role	Active participant in care decisions	Recipient of divine and medical healing
Nurse's Role	Facilitator of goal-setting	Spiritual and physical caregiver
Environment	Social and interpersonal systems	Faith-based healing environment
Goal of Care	Achieve mutually set health objectives	Restore wholeness through God's design

3. Application in Nursing Practice

Nursing Process Step	King's Approach	Adventist Approach
Assessment	Focus on patient's perceptions and needs	Evaluate body, mind, and spiritual health
Planning	Collaborative goal-setting (e.g., pain management)	Integrate prayer, nutrition, and medical care
Implementation	Nurse-patient partnership in interventions	Blend evidence-based care with faith practices
Evaluation	Measure goal attainment	Assess holistic healing (physical + spiritual)

4. Strengths and Limitations

theory	strengths	Limitations
King's Theory	Structured, practical framework Empowers patient autonomy	Less emphasis on spirituality May overlook cultural/faith factors
Adventist	-Integrates faith and science	May not suit secular settings

Model	Preventive, whole-person focus	Requires adaptation for non-Christian patients
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5. Key Similarities and Differences

Similarities:

1. Both emphasize on patient-centered care
2. Both recognize the importance of environment both social and spiritual.
3. Both use goal-directed interventions that is clinical and faith-based goals

Differences:

1. King focuses on interpersonal communication while Adventist model prioritizes divine healing.
2. Adventist care includes lifestyle medicine e.g., plant-based diet whereas King's theory is neutral on specific health practices.

Reflection

How Theories Support Holistic Care

1. King's Theory: Strengthens physical and psychological care through structured collaboration.
2. Watson's Theory: Enhances emotional and spiritual well-being through compassion.

3. Adventist Model: Unifies all dimensions under a faith-based framework.

Conclusion

While King's theory provides a systematic approach and Watson's theory fosters emotional-spiritual connections, the Adventist model integrates both into a biblically grounded practice.

Nurses can combine these theories to deliver comprehensive, patient-centered care that addresses body, mind, and spirit.

References

1. King, I. M. (1981). *A theory for nursing: Systems, concepts, process*. Wiley.
2. Watson, J. (2008). *Nursing: The philosophy and science of caring* (Rev. ed.). University Press of Colorado.
3. Jones, R. T., Ramal, E. M., & Petersen, S. L. (2023). *Whole person care: A holistic approach to health and healing*. Springer Publishing.